MERIDIAN BAY MEDICAL CENTER

FICTIONAL DOCUMENT — FOR SOFTWARE TESTING ONLY. Not real medical guidance.

Patient: Daniel Okonkwo, 58-year-old male
Discharge date: March 11, 2026

1. Reason for Admission

Mr. Okonkwo presented with progressive dyspnea on exertion, intermittent palpitations, and one episode of chest tightness. Initial ECG demonstrated atrial fibrillation with rapid ventricular response.

4. Discharge Medications

The following regimen was prescribed at discharge.

Medication	Dose	Route / Frequency	Duration	Indication
Velantine	10 mg	PO once daily	Continue indefinitely	Hypertension
Cordizem-XR	100 mg	PO once daily	Continue indefinitely	Rate control, hypertension
Pravoxil	20 mg	PO every morning	Continue indefinitely	Hyperlipidemia
Etrazolam	0.5 mg	PO twice daily	8 weeks	Acute anxiety, sleep disturbance
Pranixol	10 mg	PO once daily	Continue indefinitely	Adjunctive cardiovascular protection
Quintaxin	20 mg	PO once daily, before breakfast	8 weeks, reassess	GERD prophylaxis

5. Counseling and Warnings Reviewed with Patient

• Pravoxil should be taken in the morning with breakfast for best tolerability.

• Quintaxin should be taken thirty minutes before breakfast for optimal effect.

6. Follow-Up

The patient was instructed to follow up with primary care within seven days of discharge.